

PROVIDER ATTESTATION

Generated Date: 07/27/2026

I, **Ally Zillman, MD**, 1910005773, attest that I am the rendering and ordering physician for **Edward C McPherson** (DOB **12/08/1953**) for the real-time telehealth encounter on July 27, 2026. I personally performed this encounter and verified the patient's identity.

I certify that:

- Verbal informed consent for this telehealth visit and for each service ordered was obtained and recorded by staff prior to and during the encounter; no written or electronic patient signature is collected per our verbal-consent policy, and the recorded responses are documented in the encounter's Consent component.
- The medical record entry for this encounter accurately reflects my signature and notations.
- I have reviewed and attest to the accuracy of the companion documents generated for this encounter: Clinical Encounter Notes, Progress Notes, Consent component, Problem List/Diagnosis List, Medication List (if applicable), Family History, Medical Necessity letter, and Physician Order for laboratory testing.
- The diagnosis codes (Z71.89, D84.9, Z82.61, Z82.69, Z83.49, Z00.01, R30.0, N40.0, M05.49, I10, Z13.31, N23) are supported by my clinical assessment as documented in the Clinical Encounter Notes and Progress Notes.
- I ordered the following during this encounter: UTI (Urinary Tract Infection (UTI)) - UTI W/ ABR REFLEX. The clinical indication and coverage justification for each are documented in the corresponding Physician Order and Medical Necessity letter.

The ordered service is medically necessary for **Edward C McPherson** based on my clinical assessment of polyuria, dysuria, urinary frequency and urgency, cloudy and strong-smelling urine, incomplete bladder emptying sensation, difficulty initiating urination with weak stream, benign prostatic hyperplasia without lower urinary tract symptoms, and unspecified renal colic, in the context of his age, essential hypertension, immunodeficiency, and abnormal findings on general adult medical examination. These factors raise concern for urinary tract infection and related urinary pathology requiring diagnostic evaluation via UTI (Urinary Tract Infection (UTI)) - UTI W/ ABR REFLEX to guide appropriate management and antimicrobial stewardship. The full justification is provided in the Medical Necessity letter component for this encounter.

Physician Signature:



Signed: 07/27/2026

CLINICAL ENCOUNTER NOTES

Generated Date: 07/27/2026

1 | PATIENT IDENTIFICATION

Name: E
DOB: 12
Address:
Phone: (

2 | TREATING PHYSICIAN

Na
NP
Pr
Ac Jordan, UT, 84084

3 | CLINICAL ENCOUNTER NOTE

Date of Service: 07/27/2026
Encounter Type: Wellness Lite
Consult Duration: 09:31
Established Treating Relationship:
This is an initial encounter to establish care.

History of Present Illness (HPI):
Patient is a 72-year-old Male with a history of unspecified immunodeficiency, dysuria, benign prostatic hyperplasia without lower urinary tract symptoms, rheumatoid arthritis with associated myopathy involving multiple sites, essential hypertension, and unspecified renal colic. Relevant family history includes a grandmother with diabetes.

Family Medical History:
• The patient's Grandmother had a history of diabetes (deceased).

Current Medications:
• No current medications reported

Measurements:

Height: 5	Weight: 2	BMI: 3
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Vitals:

Blood Pressure: N/A	Pulse Rate: N/A	Oxygen Saturation: N/A
Blood Sugar: N/A	Temperature: N/A	

4 | ASSESSMENT & SUPPORTING CLINICAL FINDINGS (ICD-10)

The medical record supports the following codes based on the HPI above:

ICD-10	Description	Supporting Clinical Finding
Z71.89	Other specified counseling	During this wellness-lite visit, counseling was provided regarding the patient's overall very good self-reported health status, weight/BMI (5'4" and 200 lb, BMI 34.33), and urinary symptoms including polyuria.
D84.9	Immunodeficiency, unspecified	There are no reported recent hospitalizations, acute infections, or abnormal labs documented in this encounter to specifically support an underlying immunodeficiency; no objective immune-related findings are described. N/A
Z82.61	Family history of arthritis	The chart lists a family history of arthritis, although specific relatives and details are not provided in the available history for this visit.
Z82.69	Family history of other diseases of the musculoskeletal system and connective tissue	The problem list includes family history of other musculoskeletal and connective tissue diseases, consistent with documented familial arthritis-type conditions, though specific relatives are not detailed in this encounter.
Z83.49	Family history of other endocrine, nutritional and metabolic diseases	The patient reports a grandmother with diabetes (deceased), which supports a family history of other endocrine, nutritional, and metabolic diseases.
Z00.01	Encounter for general adult medical examination with abnormal findings	This visit is a wellness-lite encounter for a general adult medical examination, and abnormal findings include obesity (BMI 34.33 at 5'4" and 200 lb) and urinary complaints including polyuria and dysuria.
R30.0	Dysuria	The patient presents with urinary complaints, including dysuria as coded, in the context of polyuria noted in the presenting symptoms.
N40.0	Benign prostatic hyperplasia without lower urinary tract symptoms	Benign prostatic hyperplasia without lower urinary tract symptoms is listed on the problem list; the patient has urinary complaints (polyuria and dysuria), but no specific obstructive lower urinary tract symptoms such as hesitancy or weak stream are documented in this encounter.
M05.49	Rheumatoid myopathy with rheumatoid arthritis of multiple sites	Rheumatoid myopathy with rheumatoid arthritis of multiple sites is included in the diagnosis list as a chronic condition, though no joint pain, weakness, or musculoskeletal exam findings are specifically described in today's note.
I10	Essential (primary) hypertension	Essential (primary) hypertension is listed as an active diagnosis; while specific blood pressure values are not documented in the provided vitals, it is carried forward as a chronic condition under management.
Z13.31	Encounter for screening for depression	As part of this wellness-lite preventive visit, the patient is seen for routine screening, including depression screening, although no depressive symptoms are reported and the patient describes stress level as "never."
N23	Unspecified renal colic	Unspecified renal colic is listed among the diagnoses; however, there is no documentation in this encounter of flank pain, hematuria, or imaging results to further characterize renal colic. N/A

Physician Signature:



Signed: 07/27/2026

LABORATORY ORDER

Generated Date: 07/27/2026

1 | LABORATORY TEST ORDER

Performing Lab: Knucks

CLIA: N/A

Test Panel: UTI (Urinary Tract Infection (UTI)) - UTI W/ ABR REFLEX

Test Parameters:

Escherichia coli, Klebsiella oxytoca, Proteus species *4, Morganella morganii, Enterobacter cloacae, Klebsiella aerogenes, Klebsiella pneumoniae, Enterococcus species *3, Pseudomonas aeruginosa, Streptococcus agalactiae, Staphylococcus saprophyticus

Specimen: Buccal Swab

2 | PATIENT INSURANCE INFORMATION

Payer: [REDACTED]

Policy: [REDACTED]

Physician Signature:

[REDACTED]

Signed:

LETTER OF MEDICAL NECESSITY

Generated Date: 07/27/2026

1 | ETIOLOGY & RISK

[REDACTED] Edward C. McDharris presents with polyuria, which can be associated with urinary tract pathology and warrants evaluation for infection as a potential reversible cause. The ordered UTI (Urinary Tract Infection (UTI)) - UTI W/ ABR REFLEX is clinically necessary to assess for a urinary tract infection that could be contributing to his urinary symptoms. Given his diagnoses of immunodeficiency, unspecified, and other specified counseling, prompt identification of a UTI is important to reduce the risk of complications in the setting of potentially impaired host defenses. His family history of arthritis is noted but does not alter the immediate need to rule out or confirm UTI as a cause of his current symptoms.

2 | IMPACT ON TREATMENT & MANAGEMENT

UTI (Urinary Tract Infection (UTI)) - UTI W/ ABR REFLEX is expected to influence treatment of the condition toward which the testing is directed.

Per my evaluation on July 27, 2026, this ordered service is medically necessary to manage the beneficiary's specific medical problem (Polyuria) in the following ways:

Medication Management:

N/A

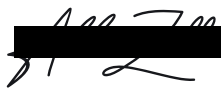
Avoidance of Futile Procedures:

N/A

Prognostic Management:

N/A

Physician Signature:


[REDACTED]

Signed:

PROGRESS NOTE

Generated Date : Jul 27 2026

1 | MEDICAL HISTORY

PATIENT IDENTIFICATION INFORMATION

Medical Record: [REDACTED]
Patient Name: Ed [REDACTED]
DOB: Dec 03 1956 (Age 69)
Sex: Male

GENERAL INFORMATION

Provider: [REDACTED]
Date of Service: [REDACTED]
Type of Visit: [REDACTED]
Location: T [REDACTED]

Current Medical Complaint:

None [REDACTED]

Past Medical History (PMHx)

- bph renal colic ra htn

Past Surgical History (PSHx)

Reviewed with patient; no active or notable history reported.

Exam

Digital recording of consult on file.

Provider History

Reviewed with patient; no active or notable history reported.

Vitals

I have checked this section with the patient. No discernible medical findings within this section.

2 | MEDICATIONS

Are you currently taking any prescription medications, over-the-counter drugs, or supplements?

☐ Yes ☒ No

Current Medications

Reviewed with patient; no active or notable history reported.

Side Effects

Reviewed with patient; no active or notable history reported.

Do you have any allergies?

☐ Yes ☒ No ☐ No known allergies (NKA)

Allergies

Reviewed with patient; no active or notable history reported.

3 | REVIEW OF SYSTEMS

Constitutional

☐ Fever ☐ Chills ☐ Fatigue ☐ Anorexia ☐ Polydipsia ☐ Polyphagia ☐ Night Sweats ☐ Unintentional Weight Loss

Respiratory

☐ Cough ☐ Dyspnea ☐ Wheezing

Cardiovascular

☐ Chest Pain ☐ Palpitations ☐ Dyspnea At Rest ☐ Lower Extremity Edema

Gastrointestinal

☐ Nausea ☐ Diarrhea ☐ Vomiting ☐ Constipation ☐ Abdominal Pain

Urinary

☐ Dysuria ☒ Polyuria ☐ Hematuria ☐ Urge Symptoms ☐ Urinary Incontinence

Dermatologic / Integumentary

☐ No Lesions ☐ Change In Skin Texture

Musculoskeletal

☐ Back Pain ☐ Muscle Pain ☐ Muscle Cramps ☐ Difficulty Walking

Neurological

☐ Numbness ☐ Tingling ☐ Headaches ☐ Lightheadedness

Psychiatric

☐ Anxiety ☐ Depression

Hematologic / Lymphatic

☐ Easy Bruising

Review of Systems Notes

4 | FAMILY HISTORY

Do you have any family medical history you would like to note? ☒ Yes ☐ No

Family Member	Condition	Maternal side / Paternal side	Status
Grandmother	diabetes	Maternal side	Deceased

5 | GENERAL HEALTH

General Well-Being

Which of the following best describes your current living arrangement? ☐ Single ☒ Married ☐ Widowed

How would you rate your overall health? ☐ Excellent ☒ Very Good ☐ Good ☐ Fair ☐ Poor

Have you been hospitalized in the past 12 months?

☐ Yes ☒ No

Nutrition

On a special diet?

☐ Yes ☒ No

Happy with your current weight?

☒ Yes ☐ No

Within the past 12 months, did you worry your food would run out before you got money to buy more?

☐ Yes ☒ No

Within the past 12 months, did the food you bought not last and you didn't have money to get more?

☐ Yes ☐ No

Physical Activity

How many days a week do you usually exercise?

☐ 0 times ☐ 1–2 times ☐ 3–4 times ☒ 5–7 times

On days when you exercise, for how long do you usually exercise? **walk**

How intense is your typical exercise?

☐ I'm currently not exercising ☐ Light (stretching or slow walking) ☒ Moderate (like brisk walking) ☐ Heavy (like jogging or swimming)
☐ Very heavy (fast running, or stair climbing)

Sleep

How many hours of sleep do you get at night?

☐ 1–4 hours ☒ 5–8 hours ☐ 10 hours or more

Nutrition, physical activity, and sleep reviewed; patient lifestyle discussed.

Stress & Psychosocial

How often is stress a problem for you?

☒ Never ☐ Rarely ☐ Sometimes ☐ Usually ☐ Always

How well do you handle stress?

☐ Copes effectively ☐ At times problems coping ☐ Often problems coping

How often do you get the social/emotional support you need?

☐ Never ☐ Rarely ☐ Sometimes ☒ Usually ☐ Always

In general, how satisfied are you with your life ?

☐ Very satisfied ☐ Satisfied ☐ Neutral ☐ Dissatisfied

Pain Screening

During the past 4 weeks, how much bodily pain have you generally had?

☐ 0 — No pain: generally pain-free ☒ 1 — Tolerable: present, does not prevent activities ☐ 2 — Tolerable: prevents some activities
☐ 3 — Intolerable: can use phone, watch TV, read ☐ 4 — Intolerable: cannot use phone, watch TV, read
☐ 5 — Intolerable: unable to verbally communicate

Pain scale score:

1

6 | DEPRESSION SCREENING

PHQ-2

1. Little interest or pleasure in doing things

☒ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

2. Feeling down, depressed, or hopeless

☒ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

3. Do you have ongoing care with your primary care physician?

☐ Yes ☐ No

PHQ-2 score:

0

PHQ-9 Assessment

PHQ-9 Item	Score
1. Little interest or pleasure in doing things	☐ 0 ☐ 1 ☐ 2 ☐ 3
2. Feeling down, depressed, or hopeless	☐ 0 ☐ 1 ☐ 2 ☐ 3
3. Trouble sleeping	☐ 0 ☐ 1 ☐ 2 ☐ 3
4. Feeling tired or having low energy	☐ 0 ☐ 1 ☐ 2 ☐ 3
5. Poor appetite or overeating	☐ 0 ☐ 1 ☐ 2 ☐ 3
6. Feeling bad about yourself	☐ 0 ☐ 1 ☐ 2 ☐ 3
7. Trouble concentrating	☐ 0 ☐ 1 ☐ 2 ☐ 3
8. Moving or speaking slowly, or being restless	☐ 0 ☐ 1 ☐ 2 ☐ 3
9. Thoughts of self-harm	☐ 0 ☐ 1 ☐ 2 ☐ 3

PHQ-9 total score:

0

If problems were checked, difficulty in daily functioning ☒ Not difficult ☐ Somewhat difficult ☐ Very difficult ☐ Extremely difficult

Depression screening completed using a standardized, validated tool. Results reviewed and addressed as appropriate.

7 | ALCOHOL SCREENING

Do you drink alcohol? ☐ Yes ☒ No

8 | TOBACCO USE SCREENING

Do you currently smoke? ☐ Yes ☒ No

Tobacco use status reviewed and updated.

9 | SUBSTANCE USE

Do you use recreational drug? ☐ Yes ☒ No

Patient denies illicit drug or non-prescribed substance use.

10 | PREVENTIVE SCREENING AND VACCINATION

Screenings — completed in the past year?

Exam / Test	Status	Date
Eye Exam	☒ Yes ☐ No	—

Exam / Test	Status	Date
Hearing Exam	☒ Yes ☐ No	—
Colonoscopy / Sigmoid	☒ Yes ☐ No	—
Bone Density	☐ Yes ☒ No	—
Prostate Exam / PSA Level	☒ Yes ☐ No	—
Mammogram	☐ Yes ☒ No	—
Abdominal Aortic Aneurysm	☐ Yes ☒ No	—

Preventive screening history reviewed, including age-appropriate cancer screenings and diagnostic exams.

Immunizations — updated in the past year?

Vaccine	Status	Date
Influenza (Flu)	☒ Yes ☐ No	—
Tetanus Booster or Tdap	☒ Yes ☐ No	—
Zoster Vaccine (Shingles)	☒ Yes ☐ No	—
Pneumococcal (Pneumonia)	☒ Yes ☐ No	—
Hepatitis B	☒ Yes ☐ No	—
COVID-19	☒ Yes ☐ No	—

Immunization status reviewed and updated according to CDC and Medicare preventive guidelines.

11 | FUNCTIONAL STATUS & SAFETY

A. Activities Of Daily Living (ADLs)

ADL	Level of Assistance
Bathing	☒ No Help ☐ Some Help ☐ Total Help
Dressing	☒ No Help ☐ Some Help ☐ Total Help
Eating	☐ No Help ☐ Some Help ☐ Total Help
Toileting	☒ No Help ☐ Some Help ☐ Total Help
Walking / Transferring	☒ No Help ☐ Some Help ☐ Total Help

B. Instrumental Activities Of Daily Living (IADLs)

IADL	Level of Assistance
Meal preparation	☒ No Help ☐ Some Help ☐ Total Help

IADL	Level of Assistance
Shopping	☒ No Help ☐ Some Help ☐ Total Help
Housekeeping	☒ No Help ☐ Some Help ☐ Total Help
Managing meds	☒ No Help ☐ Some Help ☐ Total Help
Transportation	☐ No Help ☐ Some Help ☐ Total Help

Functional status and social support reviewed. Patient reports adequate support and no difficulty with ADLs/IADLs.

C. Fall Risk

Any falls in the past year? ☐ Yes ☒ No

Are you afraid of falling? ☐ Yes ☒ No

Do you use assistive devices? ☒ None ☐ Cane ☐ Walker ☐ Wheelchair ☐ Other

Fall risk assessment completed, including history of falls, fear of falling, and use of assistive devices.

D. Home Safety

Do you live in a home or apartment? ☒ Home ☐ Apartment

In the last 12 months have you had any problems with getting heat, water or electricity? ☐ Yes ☒ No

Do you have grab bars in the bathroom, handrails on the stairs, or non-slip mats? ☒ Yes ☐ No

Do you feel safe at home? ☒ Yes ☐ No

Do you feel safe where you live? ☒ Yes ☐ No

In the last 12 months, have you been hit, slapped or kicked by someone in your home? ☐ Yes ☒ No

Have you been emotionally disgraced or bullied by your partner / roommate? ☐ Yes ☒ No

E. Motor Vehicle Safety

Do you have the ability to drive your car? ☒ Yes ☐ No ☐ N/A

Do you fasten your seat belt when you are in the car? ☒ Yes ☐ No ☐ N/A

Do you ever drive after drinking or ride with a driver who has been drinking? ☐ Yes ☒ No ☐ N/A

Within the past 12 months, has lack of transportation kept you from going to medical appointments, getting your medicines, non-medical meetings or appointments, work, or from getting the things you need (food)? ☐ Yes ☒ No ☐ N/A

Home safety, motor vehicle safety, and environmental risk factors were reviewed.

12 | ADVANCE CARE PLANNING

Do you have an advance directive or living will? ☒ Yes ☐ No

Code status ☐ DNR ☐ Comfort Care ☒ Unknown / pending

Do you have a medical power of attorney? ☒ Yes ☐ No

Is blood transfusion acceptable in an emergency? ☒ Yes ☐ No

Do you have a patient advocate?

☒ Yes ☐ No

If yes, what is your relationship to the advocate?

☐ Daughter ☐ Son ☐ Friend ☐ Parent ☐ Sibling ☒ Spouse/Partner ☐ Grandparent ☐ Grandchild ☐ Other relative

Advance care planning status reviewed. Advance directives discussed per patient preference.

13 | ASSESSMENT & PLAN

Assessment

ICD-10	Description	Supporting Clinical Finding
Z71.89	Other specified counseling	During this wellness-lite visit, counseling was provided regarding the patient's overall very good self-reported health status, weight/BMI (5'4" and 200 lb, BMI 34.33), and urinary symptoms including polyuria.
D84.9	Immunodeficiency, unspecified	There are no reported recent hospitalizations, acute infections, or abnormal labs documented in this encounter to specifically support an underlying immunodeficiency; no objective immune-related findings are described. N/A
Z82.61	Family history of arthritis	The chart lists a family history of arthritis, although specific relatives and details are not provided in the available history for this visit.
Z82.69	Family history of other diseases of the musculoskeletal system and connective tissue	The problem list includes family history of other musculoskeletal and connective tissue diseases, consistent with documented familial arthritis-type conditions, though specific relatives are not detailed in this encounter.
Z83.49	Family history of other endocrine, nutritional and metabolic diseases	The patient reports a grandmother with diabetes (deceased), which supports a family history of other endocrine, nutritional, and metabolic diseases.
Z00.01	Encounter for general adult medical examination with abnormal findings	This visit is a wellness-lite encounter for a general adult medical examination, and abnormal findings include obesity (BMI 34.33 at 5'4" and 200 lb) and urinary complaints including polyuria and dysuria.
R30.0	Dysuria	The patient presents with urinary complaints, including dysuria as coded, in the context of polyuria noted in the presenting symptoms.
N40.0	Benign prostatic hyperplasia without lower urinary tract symptoms	Benign prostatic hyperplasia without lower urinary tract symptoms is listed on the problem list; the patient has urinary complaints (polyuria and dysuria), but no specific obstructive lower urinary tract symptoms such as hesitancy or weak stream are documented in this encounter.
M05.49	Rheumatoid myopathy with rheumatoid arthritis of multiple sites	Rheumatoid myopathy with rheumatoid arthritis of multiple sites is included in the diagnosis list as a chronic condition, though no joint pain, weakness, or musculoskeletal exam findings are specifically described in today's note.
I10	Essential (primary) hypertension	Essential (primary) hypertension is listed as an active diagnosis; while specific blood pressure values are not documented in the provided vitals, it is carried forward as a chronic condition under management.
Z13.31	Encounter for screening for depression	As part of this wellness-lite preventive visit, the patient is seen for routine screening, including depression screening, although no depressive symptoms are reported and the patient describes stress level as "never."
N23	Unspecified renal colic	Unspecified renal colic is listed among the diagnoses; however, there is no documentation in this encounter of flank pain, hematuria, or imaging results to further characterize renal colic. N/A

History Of Present Illness (HPI)

Patient [REDACTED] has a history of unspecified immunodeficiency, dysuria, benign prostatic hyperplasia without lower urinary tract symptoms, rheumatoid arthritis with associated myopathy involving multiple sites, essential hypertension, and unspecified renal colic. Relevant family history includes a grandmother with diabetes.

Care Plan

Rheumatoid myopathy with rheumatoid arthritis of multiple sites · M05.49

Acute · Active

- **Plan:** Continue treatment with a prednisone taper as previously prescribed.
- **Goal:** Follow up with rheumatology as needed for pain management.

General Care

Chronic · Inactive

- **Plan:** Continue current medications for hypertension and benign prostatic hyperplasia as prescribed.
- **Goal:** The patient will use medications as directed.

Orders — Lab

Test Name	Lab	Reason for Exam	Relevant Diagnosis
UTI W/ ABR REFLEX	Knucks		Z71.89, D84.9, Z82.61, Z82.69, Z83.49, Z00.01, Z13.89, R30.0, N40.0, M05.49, I10, Z13.31, N23

Physician Signature :

[REDACTED]

Signed : Jul 27 2026

SOAP NOTE

Generated Date: 07/27/2026

Medical History

Patient Name

DOB: 12/

Date of Service: 07/27/2026

Provider

Type

S | SUBJECTIVE

Medical History

- [REDACTED] is presenting with polyuria, characterized by a frequent or urgent need to urinate, cloudy and strong-smelling or bloody urine, a sensation of incomplete bladder emptying, and difficulty starting urination with a weak urine stream.
- The provider notes that an extensive personal and family history was obtained and that the patient meets criteria to benefit from genetic testing; the benefits and self-administration instructions were explained, and the patient wishes to proceed with testing.
- He rates his overall health as very good.
- He reports typically obtaining 5–8 hours of sleep per night.
- He reports that stress is never a problem for him and that he is usually able to cope effectively with any stress in his life.
- He exercises 5–7 times per week, primarily walking, and describes his typical exercise intensity as moderate, similar to brisk walking.
- He reports tolerable bodily pain over the past 4 weeks that is present but does not prevent any activities.
- He denies being on a special diet and denies any food insecurity in the past 12 months.
- He reports no hospitalizations in the past 12 months.
- He reports a past medical history of benign prostatic hyperplasia (BPH), renal colic, rheumatoid arthritis (RA), and hypertension (HTN).
- He reports difficulties driving his car.
- He denies any falls in the past year and denies fear of falling.
- He reports that he does not use assistive devices and that he has grab bars in the bathroom, handrails on the stairs, or non-slip mats at home.
- He reports that he feels safe at home and that he always fastens his seat belt when in a car.
- He reports that he is married.
- He reports that he usually receives the social and emotional support he needs.
- He reports that he does not drink alcohol.
- He reports that he does not currently smoke and denies recreational drug use.
- Tobacco use status was reviewed and updated, and the patient denies tobacco use.
- Depression screening was completed using a standardized, validated tool, and any results were reviewed and addressed as appropriate; the patient reports that any problems, if present, have not made daily functioning difficult.
- He reports that he is independent with activities of daily living, including bathing, dressing, toileting, and walking/transferring, with no assistance required as documented.
- He reports that he does not require help with instrumental activities of daily living including meal preparation, shopping, housekeeping, and managing medications, with transportation assistance not specified.
- He reports having an advance directive or living will, a medical power of attorney, and a patient advocate, and he states that blood transfusion is acceptable in an emergency.

- He reports having had an eye exam, hearing exam, colonoscopy/sigmoidoscopy, and prostate exam/PSA level within the past year.
- He reports that his immunizations have been updated within the past year, including influenza (flu), tetanus booster or Tdap, zoster (shingles), pneumococcal (pneumonia), hepatitis B, and COVID-19 vaccines.
- No current outpatient medications are documented in the provided data.
- No known drug allergies (NKDA) documented.

Family History

- Family history is notable for a grandmother with diabetes who is deceased.

O | OBJECTIVE

- Anthropometric measurements: height 5 feet 4 inches, weight 200 pounds, with a calculated body mass index (BMI) of 34.33, consistent with obesity class I.
- No laboratory results are documented for this encounter.
- No imaging studies are documented for this encounter.
- The encounter was documented as a Wellness Lite visit on 2026-07-27 with a recorded duration of 09:31.
- Depression screening was completed using a standardized, validated tool (PHQ-9); results were reviewed and addressed as appropriate, and the patient reports that any symptoms, if present, have not made daily functioning difficult.
- Tobacco use status was reviewed and updated, and the patient is documented as a non-smoker.
- The patient reports no falls in the past year, no fear of falling, and no use of assistive devices, and he has home safety features such as grab bars, handrails, or non-slip mats.
- Preventive care status includes eye exam, hearing exam, colonoscopy/sigmoidoscopy, and prostate exam/PSA level performed within the past year, with bone density testing and abdominal aortic aneurysm screening not performed.
- Immunizations are up to date within the past year for influenza, tetanus booster or Tdap, zoster (shingles), pneumococcal (pneumonia), hepatitis B, and COVID-19.
- Level of medical decision making for this encounter is documented as high.
- A digital recording of the consult is on file.

A | ASSESSMENT

- Other specified counseling (Z71.89)
- Immunodeficiency, unspecified (D84.9)
- Family history of arthritis (Z82.61)
- Family history of other diseases of the musculoskeletal system and connective tissue (Z82.69)
- Family history of other endocrine, nutritional and metabolic diseases (Z83.49)
- Encounter for general adult medical examination with abnormal findings (Z00.01)
- Dysuria (R30.0)
- Benign prostatic hyperplasia without lower urinary tract symptoms (N40.0)
- Rheumatoid myopathy with rheumatoid arthritis of multiple sites (M05.49)
- Essential (primary) hypertension (I10)
- Encounter for screening for depression (Z13.31)
- Unspecified renal colic (N23)

P | PLAN



Genetics Medical Group PLLC

7533 S CENTER VIEW CT STE R, West
Jordan, UT, 84084

Date of Service: 07/27/2026

Patient [REDACTED]

Reference ID: CA-BUQVSPZF

Acute · Active

- Plan: Continue treatment with a prednisone taper as previously prescribed.
- Goal: Follow up with rheumatology as needed for pain management.

General Care

Chronic · Inactive

- Plan: Continue current medications for hypertension and benign prostatic hyperplasia as prescribed.
- Goal: The patient will use medications as directed.

Physician Signature:

[REDACTED SIGNATURE]

Signed: 07/27/2026